

Confrontation depends on fixation

Test the visual field while the patient looks centrally, not at the moving target.

A valid comparison

Sit at the same eye height and test one patient eye against the corresponding examiner field.



Ask the patient to fixate on the examiner's eye; re-establish fixation if it is lost.



Present targets midway and assess central and peripheral areas in the quadrants.



Map the blind spot with disappearance and reappearance of a small target.

Know what a normal blind spot means

The physiological blind spot lies in the temporal field, slightly below the horizontal meridian. Map its margins rather than confusing it with a broad field defect. A patient who turns toward each stimulus can appear to see targets in every quadrant despite a defect; the result is only meaningful if central fixation and the testing geometry remain controlled.

Amsler asks about central distortion

An abnormal line pattern matters even when the central dot and corners are visible.

Set up and interpret the grid

Step / report	Meaning
Reading distance with near correction	A presbyopic patient needs the appropriate reading spectacles.
One eye at a time; fixate the central dot	Avoid binocular compensation and scanning.
Wavy or bent lines	Central distortion is an abnormal result.
Missing or blurred patches	Document the affected area and arrange further assessment.

Use the grid for the right task

Ask about the central dot, corners, missing areas and whether the lines remain straight. Keep fixation on the dot while the patient reports changes elsewhere. Amsler tests central visual function; it does not establish normal peripheral fields or identify the cause of an abnormal result on its own.